Caprini VTE Risk (surgical) + SA STG VTE Prophylaxis

adult surgical / orthopaedic patients · ward / theatre

Caprini

RISK FACTORS — POINTS

Pts	Factors
1	Age 41–60 · minor surgery · major surgery <1 month · varicose veins · IBD · swollen legs · BMI >25 · acute MI · CCF <1 month · sepsis <1 month · serious lung disease incl. pneumonia <1 month · abnormal PFT/COPD · medical patient at bed rest · OCP/HRT · pregnant or postpartum · unexplained stillbirth / recurrent abortion
2	Age 61–74 · arthroscopic surgery · major open surgery >45 min · laparoscopic surgery >45 min · malignancy · confined to bed >72 h · immobilising plaster cast · central venous access
3	Age ≥75 · history of VTE · family history of VTE · factor V Leiden · prothrombin 20210A · lupus anticoagulant · anticardiolipin antibodies · ↑homocysteine · HIT · other thrombophilia
5	Stroke <1 month · elective major lower-limb arthroplasty · hip, pelvis or leg fracture · acute spinal cord injury (<1 month) · multiple trauma

RISK (TOTAL)

0
Very low

1–2
Low

3–4
Moderate

≥5
High

SA STG — PROPHYLAXIS

- Assess VTE risk (predisposing + exposing factors, Table 2.3) in every admitted patient ✚
- Enoxaparin 40 mg SC daily (20 mg if eGFR <30), or unfractionated heparin 5 000 units SC 12-hourly ✚
- Medical high risk: rivaroxaban 10 mg oral daily is an option ✚
- Orthopaedic trauma: low–moderate risk aspirin 150 mg daily; high risk enoxaparin ✚
- Hip arthroplasty: rivaroxaban 10 mg daily up to 10 days, then aspirin to 28 days ✚
- Knee arthroplasty: rivaroxaban 10 mg daily 2–7 days, then aspirin to 14 days total ✚

NOTES

- ▸ Caprini is not in the SA STG; STG uses its own predisposing/exposing risk table
- ▸ Use mechanical prophylaxis when bleeding risk is high
- ▸ Risk bands per ACCP 2012 (Gould) for general/abdominal-pelvic surgery

HOSP STG & EML Hospital Level Adults, 6th ed. 2024 (updated Mar 2026) — pp 2.24, 2.25, 2.26 · **CAPRINI** Caprini JA. Dis Mon 2005;51:70-8; Gould MK et al. Chest 2012;141(2 Suppl):e227S